

SKIN ULCER OR NON-HEALING WOUND: ROUTINE CARE

If arterial, venous or diabetic ulcer, or cause uncertain, ensure patient has been referred to district hospital for further assessment.

Assess the patient with a skin ulcer or non-healing wound

Assess	When to assess	Note
Pain	Every visit	- If pain ≥ 4 weeks, assess and advise 5 61. - If new or worsening wound pain, also check for wound infection as below. - If muscle pain in legs/buttocks on exercise that is relieved by rest, or reduced/absent foot pulses, peripheral vascular disease (PVD) likely: give routine care 5 139. If newly diagnosed with PVD, also refer same week.
Chronic conditions	As needed	Ensure patient is receiving routine care for all his/her chronic conditions. If diabetes 5 130 , hypertension 5 133 , peripheral vascular disease (PVD) 5 139 , HIV 5 111 .
Medications	Every visit	Review medication: NSAIDs (e.g. ibuprofen), prednisone and chemotherapy can cause delayed healing. Discuss with doctor.
Depression	Monthly	In the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either 5 143 .
Alcohol/drug use	First visit	In the past year, has patient: 1) drunk ≥ 4 drinks ¹ /session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any 5 142 .
Palliative care	First visit	If patient has a life-limiting illness, also consider giving palliative care 5 170 .
BMI ² and nutrition	Monthly	- If BMI < 19.5: arrange nutritional support. If BMI < 18.5 or signs of nutritional deficiency³, also refer to dietitian. - If BMI > 25: advise healthy diet and regular physical activity 5 11. If no improvement or if BMI > 30, also refer to dietitian.
CVD risk	First visit	If any CVD risk factors ⁴ , assess CVD risk 5 127 .
Wound and surrounding skin	Every visit	- Measure size and depth of wound and record in notes. - If > 10cm, circumferential (whole way round limb), visible fat/muscle/tendon/bone or increasing in size or depth, refer. - Assess colour of wound bed: is it pink (epithelialising), red (granulating), yellow (sloughy) or black (necrotic)? - Assess wound discharge: check colour, consistency, amount and smell. - If surrounding skin red/warm/swollen, or if diabetic ulcer and increasing discharge, pus or offensive smell, infection likely: - If BP $< 90/60$, pulse > 100, temperature $\geq 38^{\circ}\text{C}$, confused, blisters, crepitus⁵ or severe pain, refer urgently. - If infection extensive or getting worse despite antibiotics, undermined wound edge, ulcer/wound extends to bone, poorly controlled diabetes or previous surgery with artificial implant (like pin, plate or joint replacement), refer same day.
Blood supply to leg	If ulcer/wound on leg: every visit	- If reduced or absent foot pulses, capillary refill time > 5 seconds, difference in temperature between feet, change in skin colour, loss of hair, shiny skin, numbness or weakness, reduced blood supply likely: - If new changes, discuss urgency of referral with specialist or referral hospital. - If known with reduced blood supply and already assessed at referral hospital, continue management plan given.
Mobility	Every visit	If problem with mobility, refer to physiotherapist or occupational therapist for rehabilitation support.
Glucose	First visit	If not known with diabetes, check glucose 5 17 . If known diabetes, give routine care 5 130 .
Hb	First visit	If Hb < 12 (woman) or < 13 (man), anaemia likely 5 27 .
HIV	First visit	Test for HIV 5 110 . If HIV positive, give routine care and ensure patient is on ART 5 111 .
Wound swab	If infected and no better after 2-3 days of antibiotic	- Clean wound with sodium chloride 0.9% and leave for 5 minutes. Then, avoiding wound edges, move swab across wound bed in a zig-zag motion, while rotating swab between the fingers. If wound dry, moisten swab tip with sodium chloride 0.9% solution before taking swab. - Follow-up results to check infection has been treated appropriately.

¹One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer. ²BMI = weight (kg) $\div$ height (m) $\div$ height (m). ³Signs of nutritional deficiency include red/cracked corners of mouth, brittle/ridged nails, dry hair that falls out easily, very dry skin, muscle wasting. ⁴CVD risk factors include smoking, BMI > 25 , waist circumference > 80 cm (woman) or 94cm (man), hypertension, diabetes, cholesterol > 5.2 , parent/sibling with early onset CVD (man < 55 years or woman < 65 years).

⁵Crepitus is a crackling, popping or grating sound or feeling under the skin.